

Basic Medical Triage RAG Dataset

Page 1: Dataset Scope and Triage Contract

card_id: MED-RAG-001-GLOBAL-SAFETY | source_ids: MLP-EMERG, MLP-ER

last_reviewed_for_dataset: 2026-05-22 | audience: adult general triage; pediatric/pregnancy rules require extra caution

Purpose

- Use this dataset to help a RAG assistant answer basic medical concern questions and route users toward self-care, routine appointment, urgent same-day care, or emergency help.
- This dataset is educational support content. It is not a diagnosis tool, not a medical device, and should be reviewed by a licensed clinician before production use.
- For U.S. deployments use 911 for emergencies, 988 for suicide/crisis support, and Poison Help at 1-800-222-1222. For other countries, localize emergency numbers before release.

Triage labels

- EMERGENCY_NOW: possible threat to life, limb, pregnancy, airway, brain, heart, severe bleeding, poisoning, or immediate self-harm risk. Tell the user to call emergency services now.
- URGENT_SAME_DAY: new, worsening, moderate-to-severe, or high-risk symptoms that should be assessed by a clinician or urgent care today.
- PRIMARY_CARE_24_72H: non-emergency symptoms that need an appointment soon, especially if persistent, recurring, or affecting function.
- SELF_CARE_MONITOR: mild, short-lived symptoms with no red flags and no high-risk context. Give conservative home-care guidance and escalation signs.
- INSUFFICIENT_INFO: ask focused questions, but do not delay emergency guidance if a red flag is already present.

RAG answer contract

- Return: triage_label, short_answer, why_this_route, self_care_if_safe, appointment_timing, emergency_escalation_signs, source_ids.
- Ask for age, pregnancy or postpartum status, symptom duration, severity, major conditions, immune suppression, medicines, recent injury, fever, hydration, and breathing or mental-status changes when relevant.
- Do not reassure away severe or unusual symptoms. If uncertainty remains and the symptom could be serious, route one level higher.

Retrieve when

- Use this page with every medical query as the system safety frame and output schema.

Basic Medical Triage RAG Dataset

Page 2: Universal Red Flags and Routing

card_id: MED-RAG-002-UNIVERSAL-RED-FLAGS | source_ids: MLP-EMERG, MLP-ER, CDC-RESP

last_reviewed_for_dataset: 2026-05-22 | audience: adult general triage; pediatric/pregnancy rules require extra caution

EMERGENCY_NOW red flags

- Trouble breathing, stopped breathing, choking, severe shortness of breath, blue/gray/pale lips or skin, or inability to speak in full sentences.
- Chest pain, pressure, squeezing, or discomfort lasting more than a few minutes, especially with sweating, nausea, dizziness, fainting, jaw/arm/back pain, or shortness of breath.
- New confusion, unusual behavior, difficult to wake, fainting, loss of consciousness, seizure, or sudden inability to speak, see, walk, balance, or move one side.
- Severe abdominal pain or pressure, a stiff/hard/tender abdomen, vomiting blood, coughing blood, black/tarry stool, or severe/persistent vomiting or diarrhea.
- Bleeding that will not stop, severe burns, head/spine injury, serious allergic reaction, suspected poisoning/overdose, or attempted self-harm.

URGENT_SAME_DAY patterns

- Symptoms are new and worsening, moderate-to-severe, unusual for the person, associated with fever, dehydration, spreading infection, or significant pain.
- High-risk context: age under 3 months with fever, pregnancy or postpartum within 1 year, immune suppression, cancer treatment, transplant, sickle cell disease, diabetes, heart/lung/kidney disease, or frailty.
- The user says something feels seriously wrong, cannot function normally, or has repeated visits/recurrence without a clear plan.

PRIMARY_CARE_24_72H patterns

- Mild symptoms persist beyond expected duration, keep recurring, interfere with sleep/work, or require prescription evaluation such as possible UTI, asthma flare, chronic cough, or worsening rash.

SELF_CARE_MONITOR boundary

- Only use self-care routing when symptoms are mild, improving, short duration, and no red flags or high-risk context are present. Always include what changes should trigger care.

Basic Medical Triage RAG Dataset

Page 3: Chest Pain, Breathing, Stroke-Like Symptoms

card_id: MED-RAG-003-CARDIO-RESP-NEURO | source_ids: MLP-CHEST, MLP-EMERG, CDC-RESP, CDC-COVID
last_reviewed_for_dataset: 2026-05-22 | audience: adult general triage; pediatric/pregnancy rules require extra caution

Chest pain routing

- EMERGENCY_NOW if chest pressure, squeezing, crushing, tightening, or pain lasts more than a few minutes or spreads to jaw, left arm, back, or between shoulder blades.
- EMERGENCY_NOW if chest symptoms occur with nausea, sweating, dizziness, fainting, racing heartbeat, or shortness of breath.
- URGENT_SAME_DAY if mild chest discomfort is new, recurring, exertional, linked with palpitations, or occurs in a person with heart disease risk, even if it improves.

Breathing routing

- EMERGENCY_NOW if breathing difficulty comes on suddenly, seriously interferes with breathing or talking, occurs at rest, wakes the user from sleep, or is accompanied by chest pain, confusion, blue/gray/pale lips or skin, or fainting.
- URGENT_SAME_DAY if shortness of breath is new, worsening, associated with fever, wheezing, cough producing blood, leg swelling, or reduced activity tolerance.
- SELF_CARE_MONITOR is only appropriate for mild nasal congestion or short cough without breathing trouble, fever red flags, chest pain, or high-risk conditions.

Stroke-like or neurologic symptoms

- EMERGENCY_NOW for sudden face drooping, arm or leg weakness/numbness, trouble speaking, confusion, vision loss, severe dizziness/loss of balance, or sudden severe headache.
- Do not ask the user to wait, sleep it off, drive themselves, or schedule routine care for possible stroke or heart attack symptoms.

Example retrieval answer

- User: 'My chest feels tight and I am sweating.' Answer route: EMERGENCY_NOW. Tell them to call emergency services now and avoid driving themselves.

Basic Medical Triage RAG Dataset

Page 4: Fever, Cold, Cough, Sore Throat, Respiratory Viruses

card_id: MED-RAG-004-FEVER-RESPIRATORY | source_ids: MLP-FEVER, CDC-RESP, CDC-COVID

last_reviewed_for_dataset: 2026-05-22 | audience: adult general triage; pediatric/pregnancy rules require extra caution

EMERGENCY_NOW respiratory and fever red flags

- Difficulty breathing or shortness of breath, persistent chest or abdominal pain/pressure, persistent dizziness, confusion, inability to wake/stay awake, seizures, or not urinating.
- Fever with stiff neck, severe headache, confusion, seizure, trouble breathing, purple/non-blanching rash, or appearing very ill.
- Respiratory illness with symptoms that improve and then return or worsen with fever or worse cough should prompt medical care; route higher if breathing or mental-status red flags are present.

URGENT_SAME_DAY or provider contact

- Adult fever that stays at or keeps rising above 103 F (39.4 C), fever lasting several days, new rash/bruises, pain with urination, dehydration signs, or serious underlying illness.
- Any fever in a baby under 12 weeks or under 3 months needs urgent clinician guidance. For children, use pediatric-specific protocols, not adult-only triage.
- High-risk users with flu/COVID-like symptoms may benefit from early testing and prescription treatment; advise prompt clinician contact.

SELF_CARE_MONITOR when safe

- Mild cold symptoms, low-grade fever, sore throat, congestion, or cough can often be monitored with rest, fluids, and OTC medicines used according to labels or clinician advice.
- CDC respiratory guidance: stay home and away from others until symptoms are improving and fever has been gone for at least 24 hours without fever-reducing medicine.

RAG caution

- Do not claim symptoms distinguish COVID, flu, RSV, strep, or pneumonia by chat alone. Suggest testing or clinician evaluation when results change treatment or isolation decisions.

Basic Medical Triage RAG Dataset

Page 5: Abdominal Pain, Vomiting, Diarrhea, Dehydration

card_id: MED-RAG-005-GI-DEHYDRATION | source_ids: MLP-ABD, MLP-VOMIT, MLP-DIARRHEA, MLP-DEHYD
last_reviewed_for_dataset: 2026-05-22 | audience: adult general triage; pediatric/pregnancy rules require extra caution

Abdominal pain red flags

- EMERGENCY_NOW for sudden sharp abdominal pain, severe abdominal pain/pressure, chest/neck/shoulder pain with belly pain, hard or tender abdomen, vomiting blood, blood in stool, or inability to pass stool with vomiting.
- URGENT_SAME_DAY for persistent localized pain, worsening pain, fever with abdominal pain, repeated vomiting, pregnancy, older age/frailty, or significant medical conditions.
- PRIMARY_CARE_24_72H if mild pain lasts a week or more, recurs, or occurs with other symptoms that do not resolve.

Vomiting routing

- EMERGENCY_NOW if vomiting may be from poisoning, vomit contains blood or coffee-ground material, or vomiting occurs with severe headache/stiff neck, severe belly pain, confusion, or fainting.
- URGENT_SAME_DAY if vomiting lasts more than 24 hours, the user cannot keep fluids down for 12 hours or more, has no urination for 8 hours, or vomits 3 or more times in 1 day with concerning symptoms.

Diarrhea and dehydration routing

- URGENT_SAME_DAY for signs of dehydration, diarrhea more than 2 days in adults, fever of 102 F or higher, severe abdominal/rectal pain, blood or pus in stool, or black/tarry stool.
- EMERGENCY_NOW for dehydration with loss of consciousness, confusion, seizure, heatstroke signs, severe weakness, or worsening despite treatment.

SELF_CARE_MONITOR when safe

- Mild short-lived stomach upset without red flags can focus on fluids, oral rehydration, bland foods as tolerated, and avoiding alcohol. Escalate if red flags appear or symptoms persist.

Basic Medical Triage RAG Dataset

Page 6: Headache, Dizziness, Head Injury, Neurologic Changes

card_id: MED-RAG-006-HEADACHE-NEURO | source_ids: MLP-HEAD, MLP-EMERG, CDC-MATERNAL

last_reviewed_for_dataset: 2026-05-22 | audience: adult general triage; pediatric/pregnancy rules require extra caution

Headache **EMERGENCY_NOW** red flags

- First severe headache, sudden explosive/violent onset, 'worst headache ever,' or headache that rapidly worsens.
- Headache with slurred speech, vision change, weakness/numbness, trouble moving arms or legs, loss of balance, confusion, memory loss, fainting, or seizure.
- Headache with fever, stiff neck, nausea/vomiting; headache after head injury; severe headache in one red eye; new headache after age 50; or new headache with cancer or immune suppression.
- Pregnancy/postpartum: headache that will not go away, gets worse, is sudden/severe, or comes with blurred vision or dizziness is an urgent maternal warning sign.

Headache **PRIMARY_CARE_24_72H** or urgent patterns

- See a clinician soon if headaches wake the user from sleep, last more than a few days, are worse in the morning, become more frequent, or change in pattern/intensity.
- **URGENT_SAME_DAY** if headache is new and moderate-to-severe, associated with persistent vomiting, dehydration, pregnancy/postpartum, high blood pressure, or concerning medical history.

Dizziness and fainting

- **EMERGENCY_NOW** for fainting/loss of consciousness, dizziness with chest pain, trouble breathing, neurologic symptoms, severe headache, pregnancy/postpartum warning signs, or suspected stroke.
- **PRIMARY_CARE_24_72H** for recurrent lightheadedness without red flags, medication-related dizziness, or dizziness affecting daily activities.

RAG caution

- Never advise driving with neurologic symptoms, fainting, severe dizziness, or possible stroke/heart symptoms.

Basic Medical Triage RAG Dataset

Page 7: Urinary Symptoms, Reproductive Concerns, Pregnancy/Postpartum

card_id: MED-RAG-007-UTI-PREGNANCY | source_ids: MLP-UTI, CDC-MATERNAL, MLP-FEVER

last_reviewed_for_dataset: 2026-05-22 | audience: adult general triage; pediatric/pregnancy rules require extra caution

Possible UTI routing

- Symptoms can include burning/pain with urination, frequent urge to urinate, lower belly pressure, cloudy/bad-smelling/reddish urine, fever/tiredness/shakiness, or back/side pain below the ribs.
- PRIMARY_CARE_24_72H for typical mild urinary symptoms because UTIs often require urine testing and antibiotics.
- URGENT_SAME_DAY if urinary symptoms occur with fever, chills, flank/back pain, nausea/vomiting, pregnancy, diabetes, kidney disease, immune suppression, male sex, older adult confusion, or worsening illness.
- EMERGENCY_NOW if urinary symptoms accompany severe weakness, confusion, fainting, sepsis-like symptoms, inability to keep fluids down, or severe pain.

Pregnancy or postpartum within 1 year

- Seek medical care immediately for fever of 100.4 F (38 C) or higher, headache that will not go away or worsens, vision changes, dizziness/fainting, extreme swelling of face/hands, trouble breathing, chest pain, or fast heartbeat.
- Seek medical care immediately for severe nausea/vomiting, inability to drink for more than 8 hours or eat for more than 24 hours, severe belly pain, baby movement stopping/slowness, vaginal bleeding, fluid leaking, or foul-smelling discharge.
- Thoughts about harming self or baby are urgent. Use 988 for crisis support in the U.S.; call emergency services if there is immediate danger.

Reproductive concern boundary

- Route heavy vaginal bleeding, pregnancy bleeding with pain/dizziness, severe pelvic pain, testicular pain/swelling, or sexual assault concerns to immediate clinician/emergency support according to severity.

RAG caution

- Always ask pregnancy/postpartum status for abdominal, urinary, bleeding, headache, chest pain, shortness of breath, severe nausea, dizziness, or mental health queries.

Basic Medical Triage RAG Dataset

Page 8: Skin, Rash, Hives, Allergic Reactions

card_id: MED-RAG-008-SKIN-ALLERGY | source_ids: MLP-RASH, MLP-HIVES, MLP-ANAPH

last_reviewed_for_dataset: 2026-05-22 | audience: adult general triage; pediatric/pregnancy rules require extra caution

Allergic reaction EMERGENCY_NOW

- Anaphylaxis is a severe whole-body allergic reaction that needs immediate medical care. Route EMERGENCY_NOW for trouble breathing, wheezing, throat tightness, hoarse/whisper voice, difficulty swallowing, swollen tongue/face/throat, fainting, or severe widespread symptoms after an exposure.
- If the person has prescribed epinephrine for anaphylaxis, tell them to follow their emergency action plan and call emergency services. Do not wait to see if severe symptoms worsen.

Rash and skin infection routing

- URGENT_SAME_DAY for rash with fever, severe pain, rapidly spreading redness/swelling, pus, red streaks, warmth, tenderness, new bruising, blisters/raw skin over large areas, rash after a new medicine, or rash involving eyes/mouth/genitals.
- PRIMARY_CARE_24_72H for rash that is bad, does not go away, keeps recurring, or occurs with other symptoms.
- SELF_CARE_MONITOR may fit mild localized itchy rash or hives without breathing/swallowing problems, facial/tongue swelling, fever, severe pain, or spreading infection signs.

Hives boundary

- Hives often go away on their own, but hives with airway swelling or breathing difficulty are a medical emergency.
- Ask about new foods, medicines, insect stings, latex, infections, and prior anaphylaxis. Prior severe allergy should lower the threshold for urgent care.

RAG caution

- Do not diagnose rashes from text alone. For image-based triage, still use red flags and advise clinician review for uncertain, spreading, painful, or systemic symptoms.

Basic Medical Triage RAG Dataset

Page 9: Cuts, Bleeding, Burns, Sprains, Minor Injuries

card_id: MED-RAG-009-INJURY-FIRST-AID | source_ids: MLP-CUTS, MLP-BURNS, MLP-BLEED, MLP-EMERG

last_reviewed_for_dataset: 2026-05-22 | audience: adult general triage; pediatric/pregnancy rules require extra caution

Bleeding and wounds

- EMERGENCY_NOW for bleeding that will not stop, spurting blood, a deep wound to chest/abdomen/pelvis/groin/neck, exposed bone/tendon, loss of consciousness, or severe injury.
- URGENT_SAME_DAY for deep cuts, puncture wounds, animal/human bites, dirty wounds, foreign body in wound, numbness, impaired movement, tetanus concern, or signs of infection.
- Signs of wound infection include increasing warmth/redness, swelling, painful/throbbing sensation, pus-like drainage, fever, or red streaks spreading from the wound.

Burns

- EMERGENCY_NOW for burns with trouble breathing, airway/face burns, chemical/electrical burns, large/deep burns, burns on hands/feet/face/groin/buttocks/major joints, circumferential burns, or burns in high-risk patients.
- URGENT_SAME_DAY if pain persists, blisters are large, the burn gets more painful, has pus, causes fever, or there are concerns about infection or function.
- SELF_CARE_MONITOR only for small superficial burns without high-risk location, severe pain, blistering concern, or infection signs.

Sprains, strains, and head injury boundary

- EMERGENCY_NOW for head/spine injury with confusion, loss of consciousness, repeated vomiting, seizure, worsening headache, weakness/numbness, or neck/back neurologic symptoms.
- URGENT_SAME_DAY for deformity, inability to bear weight or use a limb, severe swelling, severe pain, numbness, open injury, or pain after significant fall/trauma.

RAG caution

- First-aid advice should not replace urgent wound closure, burn evaluation, tetanus assessment, imaging, or emergency care when red flags exist.

Basic Medical Triage RAG Dataset

Page 10: Mental Health, Poisoning, RAG Implementation Notes

card_id: MED-RAG-010-CRISIS-POISON-IMPLEMENTATION | source_ids: MLP-SUICIDE, 988, NIMH, MLP-POISON, HRSA-POISON
last_reviewed_for_dataset: 2026-05-22 | audience: adult general triage; pediatric/pregnancy rules require extra caution

Mental health crisis routing

- If someone has attempted suicide, may imminently harm themselves/others, cannot stay safe, or has a life-threatening emergency, route EMERGENCY_NOW and tell them to call emergency services.
- For suicidal thoughts, emotional distress, or concern about someone else in the U.S., call or text 988 or chat 988lifeline.org. In other countries, provide the local crisis line.
- Warning signs include talking about wanting to die, looking for a way to kill themselves, hopelessness, feeling trapped or in unbearable pain, feeling like a burden, increased substance use, agitation, recklessness, withdrawal, rage, or extreme mood swings.

Poisoning and overdose

- If poisoning, overdose, unsafe ingestion, chemical exposure, inhaled fumes, or medicine dosing error is suspected in the U.S., call Poison Help at 1-800-222-1222 or use webPOISONCONTROL.
- EMERGENCY_NOW for unconsciousness, trouble breathing, seizure, severe symptoms, intentional overdose/self-harm, or if Poison Control advises emergency care.

Recommended RAG metadata

- chunk_id, concern_area, triage_label, red_flags, safe_self_care, appointment_timing, high_risk_groups, escalation_triggers, source_ids, last_reviewed, jurisdiction_notes.
- Retrieval rule: always retrieve MED-RAG-001 and MED-RAG-002 with any symptom-specific card. If top sources disagree or confidence is low, route higher and advise clinician contact.
- Evaluation set: test chest pain, breathing trouble, fever in infant, pregnancy headache, UTI with flank pain, rash with facial swelling, suicidal intent, overdose, mild cold, and mild ankle sprain.

Source index

- Emergency baseline: MLP-EMERG <https://medlineplus.gov/ency/article/001927.htm>; MLP-ER <https://medlineplus.gov/ency/patientinstructions/000593.htm>.
- Chest and respiratory: MLP-CHEST <https://medlineplus.gov/ency/article/003079.htm>; CDC-RESP <https://www.cdc.gov/respiratory-viruses/about/index.html>; CDC-COVID <https://www.cdc.gov/covid/signs-symptoms/>.
- Fever and GI: MLP-FEVER <https://medlineplus.gov/ency/article/003090.htm>; MLP-ABD <https://medlineplus.gov/abdominalpain.html>; MLP-VOMIT <https://medlineplus.gov/ency/article/003117.htm>; MLP-DIARRHEA <https://medlineplus.gov/diarrhea.html>; MLP-DEHYD <https://medlineplus.gov/ency/article/000982.htm>.
- Head, urinary, pregnancy: MLP-HEAD <https://medlineplus.gov/ency/patientinstructions/000424.htm>; MLP-UTI <https://medlineplus.gov/ency/article/000521.htm>; CDC-MATERNAL <https://www.cdc.gov/hearher/maternal-warning-signs/index.html>.
- Skin/allergy/injury: MLP-RASH <https://medlineplus.gov/rashes.html>; MLP-HIVES <https://medlineplus.gov/hives.html>; MLP-ANAPH <https://medlineplus.gov/ency/article/000844.htm>; MLP-CUTS <https://medlineplus.gov/ency/article/000043.htm>; MLP-BURNS <https://medlineplus.gov/ency/article/000030.htm>; MLP-BLEED <https://medlineplus.gov/ency/article/000045.htm>.
- Crisis and poison: MLP-SUICIDE <https://medlineplus.gov/ency/article/001554.htm>; 988 <https://988lifeline.org/learn/warning-signs/>; NIMH <https://www.nimh.nih.gov/health/find-help/>; MLP-POISON <https://medlineplus.gov/ency/article/007579.htm>; HRSA-POISON <https://poisonhelp.hrsa.gov/about-us>.
- Review cycle: re-check sources at least quarterly and after major CDC/NIH guidance changes. Log source dates and clinician review signoff before production use.